

BEATAFED TABLET

DESCRIPTION

A 8 mm diameter, round, flat, scored on one side, plain on the other side, white tablet.

INDICATIONS

Relief of symptoms associated with the common cold, sinusitis and allergic conditions including hayfever, allergic asthma, vasomotor rhinitis, pollenosis, allergic rhinitis and urticaria.

PHARMACODYNAMICS

Pseudoephedrine is a sympathomimetic agent which acts on alpha - adrenergic receptors in the mucosa of the respiratory tract, producing vasoconstriction. The medication shrinks swollen nasal mucous membranes, reduces tissue hyperemia, edema, and nasal congestion, and increase nasal airway patency. Also, drainage of sinus secretions may be increased and obstruction eustachian ostia may be opened.

Tripolidine is an antihistamine which competes with histamine for H1 - receptor sites on effector cells. It thereby prevents responses mediated by histamine alone. The anticholinergic actions of most antihistamines provide a drying effect on the nasal mucosa.

PHARMACOKINETICS

Pseudoephedrine is absorbed from the gastro - intestinal tract. It is resistant to metabolism by monoamine oxidase and is largely excreted unchanged in the urine together with small amounts of its hepatic metabolite. It has a half - life of several hours; elimination is enhanced and half - life accordingly shorter in acid urine. Small amounts are excreted in breast milk. Tripolidine is absorbed from the gastrointestinal tract and metabolised and excreted in the urine. Half - life vary from 3 to 5 hours or more.

DOSAGE

Adult & children above 12 years: One tablet three or four times daily.

SIDE EFFECTS

Drowsiness, gastrointestinal disturbances such as nausea, vomiting, constipation and epigastric pain. Less frequent are headache, blurred vision, tinnitus, elation or depression, irritability, nightmare, anorexia, difficulty in micturition, dryness of the mouth, tightness of the chest and tingling, heaviness and weakness of the hands, photosensitivity, tachycardia, troubled breathing and increased sweating.

CONTRAINDICATIONS

Patients with severe hypertension or coronary artery disease; patients who are taking MAOIs; hepatic function impairment; predisposition to glaucoma; sensitivity to pseudoephedrine or tripolidine.

PRECAUTIONS

Beatafed may enhance the sedative effects of central depressants including alcohol, barbiturates, hypnotics, sedatives and tranquilisers. It may cause drowsiness and dulling of mental alertness; avoid driving a motor vehicle or operating heavy machinery, and abstain from alcohol. Pseudoephedrine and antihistamines should be used with caution in patients with hypertension, heart disease, diabetes, hyperthyroidism, elevated intraocular pressure and prostatic enlargement.

DRUG INTERACTIONS

Concomitant use with sympathomimetic agents eg. decongestants, tricyclic antidepressants, appetite suppressants or MOAIs may cause a rise in blood pressure and potentiate the anticholinergic and CNS depressant effects. Beatafed may also reverse the hypotensive actions of methyl dopa, alpha and beta - adrenergic blocking agents and diuretics.

TOXICOLOGY

Cross - sensitivity and / or related problems

Patients sensitive to other sympathomimetics or antihistamines may be sensitive to this medication also.

Pregnancy / Reproduction

Studies in humans have not been done.

FDA Pregnancy Category B

Breast - feeding

Pseudoephedrine and triprolidine is distributed into breast milk; use by nursing mothers is not recommended because of the higher than usual risk of side effects for infants, especially newborn and premature infants.

Paediatrics

Use is not recommended in newborn or premature infants because of the higher than usual risk of side effects.

Geriatrics

Dizziness, sedation, confusion and hypotension may be more likely to occur. They are also more susceptible to the anticholinergic side effects of antihistamines eg. dryness of mouth and urinary retention. If these side effects occur and continue or are severe, medication should probably be discontinued.

Dental

Prolonged use of antihistamines may decrease or inhibit salivary flow, thus contributing to the development of caries, periodontal disease, oral candidiasis and discomfort.

OVERDOSAGE

Sign and symptoms of overdose are clumsiness or unsteadiness, severe drowsiness, severe dryness of mouth, nose or throat, flushing or redness of face, convulsions, hallucinations, increase in blood pressure, dizziness, dry skin, shortness of breath or troubled breathing, slow or fast heartbeat, unusual nervousness, restlessness or excitement.

Recommended treatment of overdose consists of the following: Emetics and gastric lavage should be instituted within 4 hours of overdosage in order to be effective (because pseudoephedrine is rapidly absorbed from gut). Charcoal is useful only if administered within 1 hour. Saline cathartics (milk of magnesia) are sometimes used. Vasopressors to treat hypotension; however, epinephrine should not be used since it may further lower blood pressure. Forced diuresis will increase elimination of pseudoephedrine provided renal function is adequate; however, it is not recommended for severe overdosage.

The cardiac state should be monitored and serum electrolytes measured.

If there are signs of cardiac toxicity, intravenous propranolol may be indicated.

Hypokalemia may be treated with a slow infusion of a dilute potassium chloride solution.

For delirium and convulsions, intravenous diazepam may be administered.

STORAGE CONDITIONS

Keep container tightly closed.

Store in a dry place below 30°C.

Protect from light.

Keep out of reach of children.

Jauhkan daripada kanak - kanak.

SHELF LIFE

Please refer to outer package

PRESENTATION

BEATAFED TABLET	CONTENT	PACKING
	Each tablet	1 x 10's
	contains 2.5mg	2 x 10's
	Triprolidine HCl	50 x 10's
	and 60mg	
	Pseudoephedrine	
	HCl	

Not all pack sizes available locally.

Route of administration: Oral.

Product Registration Holder:

Duopharma Manufacturing (Bangi) Sdn. Bhd.

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